INFORMATION TO KEEP WITH YOU

My complete treatment handbook

Nintedanib + docetaxel

Understand your treatment. Know what to expect. Keep your plan to hand.

Displayed calendar
21 days
See the active treatment phase

Treatment stages
2
Explained in the treatment reference

Regimen source
NCCP 00372
Version 6

Day 1: docetaxel drip, with no nintedanib. Days 2–21: nintedanib capsules twice daily. Repeat every 21 days for up to 6 cycles (about 18 weeks without delays); treatment may stop earlier. A later tablet-only phase needs a new team-confirmed plan.

Treatment stages

01 **Combination phase**
Combination phase

02 **Possible tablet-only phase**
Possible tablet-only phase

Your team confirms the active stage and dates. The full treatment guide explains each step.

Inside your handbook

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My name

Hospital / oncology team

Use the urgent instructions and contact numbers in your treatment guide. Your team must replace example numbers before patient use. Your individual treatment plan takes precedence.

SACTCHECK · CHAPTER 01

Understanding your treatment

Nintedanib + docetaxel · National Cancer Control Programme (NCCP) 00372

When to get help

Emergency: call 112 / 999

Severe breathing difficulty, collapse, seizure or severe chest pain: call emergency services. Do not wait for an online answer.

Contact your oncology service immediately

Temperature 37.5°C or higher, 36°C or lower, shivering or suddenly feeling unwell — even with a normal temperature. Call your oncology team immediately for fever, shaking chills, sudden illness, breathing difficulty, chest pain, significant bleeding, severe tummy pain, uncontrolled vomiting or new severe headache, confusion, seizure or vision change. Do not wait for the next appointment.

Follow the urgent contact instructions from your treating hospital. If you cannot reach the team, seek urgent medical assessment. Outside clinic hours, use your instructed out-of-hours pathway or the emergency department.

This page does not diagnose symptoms or decide whether it is safe to stay at home. You do not need to know which medicine caused a symptom before calling.

Three medicines. Different jobs.

To help control adenocarcinoma lung cancer after first-line chemotherapy; treatment benefit and side effects are reviewed together.

TAXANE CHEMOTHERAPY

Docetaxel

Docetaxel



Stabilises microtubules

Docetaxel prevents normal rearrangement of the cell structures needed for division. It affects cancer cells and can also suppress blood cells, irritate nerves and cause fluid retention.

Stabilises microtubules → Cancer growth disrupted → Healthy tissues may also be affected

What to watch for

Fever or feeling suddenly unwell needs urgent advice. Tell the nurse immediately about infusion symptoms or pain.

ORAL ANGIOKINASE INHIBITOR

Nintedanib

Nintedanib



Blocks VEGF, FGF and PDGF receptor signalling

Nintedanib blocks several signals involved in the growth of blood vessels that support a tumour. Its effects can also cause diarrhoea, liver changes, bleeding and wound-healing problems.

Blocks VEGF, FGF and PDGF receptor signalling → Cancer growth disrupted → Healthy tissues may also be affected

What to watch for

Call early for diarrhoea or poor fluid intake; report bleeding, yellow eyes, dark urine, severe tummy pain or new neurological symptoms urgently.

The drawings are simplified explanations of drug action, not anatomical pictures or a prediction of response.

Understanding the evidence

The trial found a progression-free benefit overall and an overall-survival benefit in the prespecified adenocarcinoma subgroup.

The licensed/NCCP population is adenocarcinoma after first-line chemotherapy. Overall-population PFS and adenocarcinoma-subgroup OS are labelled separately; this does not establish an independent benefit from maintenance-only initiation.

SACTCHECK · CHAPTER 02

Your schedule, tests and appointments

Nintedanib + docetaxel · NCCP 00372

Treatment → review → next steps

Treatment stages

01 **Combination phase**
Combination phase

02 **Possible tablet-only phase**
Possible tablet-only phase

Your team confirms the active stage and dates. The full treatment guide explains each step.

This shows treatment timing, not guaranteed symptom-free days. Your team confirms the active phase and dates.

COMBINATION PHASE

Combination phase

1–6

Day 1 docetaxel; Days 2–21 nintedanib twice daily. Up to 6 cycles.

POSSIBLE TABLET-ONLY PHASE

Possible tablet-only phase

Review

Only after at least 4 completed combination cycles and team approval: nintedanib every day in 28-day cycles. This change is not automatic.

Your team confirms each stage and discusses any changes with you.

BASELINE → TREATMENT → FOLLOW-UP

My treatment and safety checks

Routine safety tests are separate from scans and tumour markers, which follow your individual plan. No universal imaging or tumour-marker interval is specified.

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00372 v6 · official regimen · Vargatef current EMA product information

THE WHOLE TREATMENT JOURNEY

Nintedanib + docetaxel: treatment and checks

Adenocarcinoma non-small-cell lung cancer (NSCLC) · combination phase

Day 1: docetaxel drip, with no nintedanib. Days 2–21: nintedanib capsules twice daily. Repeat every 21 days for up to 6 cycles (about 18 weeks without delays); treatment may stop earlier. A later tablet-only phase needs a new team-confirmed plan.

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before treatment	Treatment / review	Between visits	Phase decision	After treatment
Treatment NCCP 00372 · SMPC	Confirm combination plan	Day 1 docetaxel; Days 2–21 nintedanib	Capsules twice daily	After≥4 cycles, review possible continuation	No automatic phase switch
Blood count NCCP 00372 · SMPC	Baseline	Before each cycle	Around nadir / if unwell	After last combination as indicated	Follow abnormalities
Kidneys / liver / blood pressure (BP) NCCP 00372 · SMPC	Baseline bloods and BP	Bloods each cycle	BP as indicated; extra tests if needed	Check tolerance before change	Liver monthly if oral continuation
Scans / other care NCCP 00372 · SMPC	Individual disease plan	No fixed scan interval	Report symptoms early	Assess benefit	Agreed follow-up

What each check is for

1 **Prepare**

Before treatment

To help control adenocarcinoma lung cancer after first-line chemotherapy; treatment benefit and side effects are reviewed together.

Ask / act: Confirm the aim, phase, prescription, tests and contacts.

2

Review

Before each cycle / at agreed reviews

Check benefit and tolerability

Ask / act: Report symptoms and confirm the next plan.

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Practical treatment planning

Agree the arrangements with your oncology team.

STEP 1

Dexamethasone before docetaxel

Take the prescribed steroid course from the day before docetaxel. If missed, contact the unit before attending; do not invent a catch-up dose.

STEP 2

Infection prevention

The team should consider preventive granulocyte colony-stimulating factor (white-cell growth support) (G-CSF) injections and arrange blood counts around the low-count period. Fever needs immediate advice.

STEP 3

Capsules

Take nintedanib about 12 hours apart with food; swallow whole. Do not open, chew or crush. If missed, take the next scheduled dose only; never double up.

STEP 4

Diarrhoea plan

Have the prescribed anti-diarrhoeal and fluid plan ready. Contact oncology early for diarrhoea, poor intake or vomiting; do not wait 7 days to seek help.

STEP 5

Medicine review

Check all medicines and supplements with oncology/pharmacy. Nintedanib has P-gp interactions; docetaxel has CYP 3 A interactions, including grapefruit concerns.

STEP 6

Pregnancy prevention

Discuss fertility and effective contraception. Nintedanib requires contraception during treatment and for at least 3 months after; follow the longest advice for your complete regimen. Vomiting/diarrhoea can reduce pill absorption.

SACTCHECK · CHAPTER 03

Symptoms and supportive care

Nintedanib + docetaxel · NCCP 00372

Day 1 → Day 21: a general pattern

This is a visual orientation, not a prediction of your personal cycle. It shows when some effects often emerge and which effects are **not** tied to a particular day.

Treatment or check	Timing in this view
Docetaxel infusion	 **Day 1** Day 1 only; no nintedanib today.
Nintedanib twice daily	 **Days 2–21** Days 2–21; take only the prescribed dose.
Between-cycle checks	 **Days 1–21** Blood count around the low-count period; date agreed by the team.

Symptoms can occur at any time: Side effects and infection do not follow a guaranteed day-by-day risk curve. Contact the team when symptoms occur.

Changes for safety: Treatment may be delayed, reduced or stopped. Do not make up missed doses or change your tablets yourself.

Why these bands stay broad: Relative days without personal dates. The team confirms actual treatment and test dates. Band length does not show likelihood or severity, and the chart cannot tell you whether it is safe to receive treatment.

There are no guaranteed "safe days". Call your oncology team immediately for fever, shaking chills, sudden illness, breathing difficulty, chest pain, significant bleeding, severe tummy pain, uncontrolled vomiting or new severe headache, confusion, seizure or vision change. Do not wait for the next appointment.

Between-treatment notes

Day 1: Docetaxel day

Docetaxel infusion after checks. Do not take nintedanib today. Follow the dexamethasone prescription. Use the current prescription and contact plan.

Day 2–21: Nintedanib at home

Take the prescribed capsule dose twice daily about 12 hours apart. Continue the agreed monitoring and support plan. Use the current prescription and contact plan.

Support around the medicines

1 **Medicine review**

Questions worth asking

Who confirms my next dose?

Your treating team checks the prescription, tests and symptoms. A completed-cycle stamp records past treatment; it does not authorise the next dose.

What if the plan changes?

Ask the team to update your phase, medicines, test appointments and cycle record. Use the updated prescription.

When are my scans?

Your team sets a response and follow-up plan for your cancer and symptoms. This document does not invent a fixed scan or tumour-marker timetable.

Try explaining the plan back: Which medicines are you having now? What changes after induction? What would make you call today? Ask your team to fill any gaps.

Supportive prescription draft for review. This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Combination · docetaxel Day 1 and steroid premedication

Active medicines: Docetaxel

Emetogenic risk: Low parenteral

Medicine	Dose / route	When
Dexamethasone premedication	8 mg orally twice daily for 3 days	Start the day before docetaxel; at least 3 doses before treatment Source docetaxel premedication. If missed, consultant may consider 20 mgIV immediately before; not an automatic substitution. Do not duplicate with antiemetic steroid.

Additional phase-specific support

- Consider primary granulocyte colony-stimulating factor (white-cell growth support) (G-CSF) prophylaxis; determine need and timing from individual infection risk and local policy.
- Low-risk antiemetic reference: dexamethasone 8 mg on Day 1 and as-needed rescue according to NCIS/ local policy. The docetaxel premedication is already a steroid course: reconcile into one prescription.

- Monitor full blood count (FBC) before each cycle and around nadir; fever needs prompt assessment.
- Hypersensitivity, fluid-retention and extravasation precautions; assess neuropathy, mouth and skin toxicity.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

Combination · nintedanib Days 2–21

Active medicines: Nintedanib

Emetogenic risk: Minimal-to-low oral

No routine prophylactic antiemetic order is populated for this phase. Agree an individual symptom-treatment plan where needed.

Additional phase-specific support

- No routine oral antiemetic prophylaxis in minimal-to-low pathway; prescribe as-needed treatment after interaction/QT/constipation review.
- Give an early diarrhoea action plan with oral hydration and prescribed anti-diarrhoeal treatment; ongoing/ severe symptoms need assessment and possible interruption.
- Nintedanib is not taken on docetaxel Day 1. No double dose after a missed capsule.
- Review P-gp interactions, liver tests, blood pressure (BP), bleeding, renal status, surgery/wound healing and contraception.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

What to watch for

You do not need to know which medicine caused a symptom before calling.

1 Nerves / neurological symptoms

Both medicines

Report tingling; new severe headache, confusion, seizure or visual change is urgent.

3 Blood vessels / blood pressure

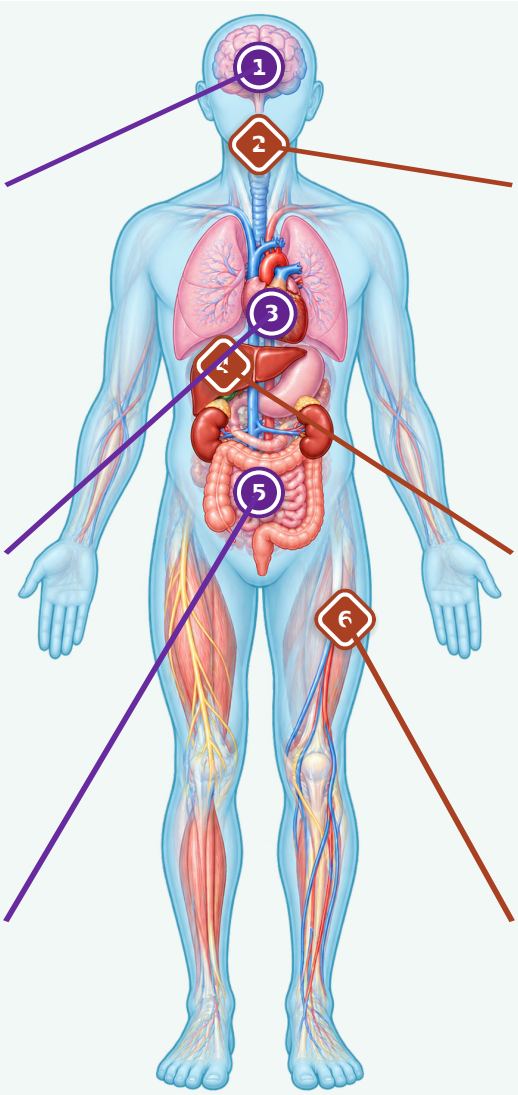
Nintedanib

Chest pain, sudden breathlessness, bleeding or a painful swollen leg needs urgent help.

5 Bowels / sickness

Both medicines

Call early for diarrhoea or vomiting; severe pain needs urgent assessment.



2 Mouth

Docetaxel

Report ulcers or pain that limits eating or drinking.

4 Liver

Nintedanib and docetaxel

Attend liver tests; report yellow skin/eyes or dark urine.

6 Blood counts / infection

Docetaxel with nintedanib

Fever, shaking chills or sudden illness needs immediate oncology advice.

Selected effects, not every possible effect or a prediction. Blood-count, skin and line markers are schematic. Number and text carry meaning; colour is decorative.

Call your oncology team immediately for fever, shaking chills, sudden illness, breathing difficulty, chest pain, significant bleeding, severe tummy pain, uncontrolled vomiting or new severe headache, confusion, seizure or vision change. Do not wait for the next appointment.

When to get help

Call 112 / 999 for severe breathing difficulty, severe chest pain, collapse or a seizure.

Call oncology immediately: temperature 37.5°C or above, 36°C or below, shivering or suddenly feeling unwell, even with a normal temperature. Follow your hospital's temperature instructions. If you cannot reach the team, seek urgent medical assessment. Do not wait for the next appointment.

Oncology daytime: 087 12345678 · **Out of hours:** 087 87654321

EXAMPLE ONLY — team must replace both numbers before patient use.

SACTCHECK · CHAPTER 04

Sources and further information

Nintedanib + docetaxel · National Cancer Control Programme (NCCP) 00372

Use this information alongside the plan agreed with your treatment team. Your team confirms your medicines, doses and appointments.

1. **National Cancer Control Programme (NCCP) 00372 v6 · official regimen**
   https://healthservice.hse.ie/documents/6913/372_Nintedanib_therapy.pdf
2. **Vargatef current EMA product information**
   https://www.ema.europa.eu/en/documents/product-information/vargatef-epar-product-information_en.pdf
3. **NCCP antiemetic classification v 6 and oral pathway**
   https://healthservice.hse.ie/documents/7152/Classification_Document_for_Systemic_Anti-_Cancer_Therapy_systemic anticancer therapy (SACT)_Induced_Nausea__pWgDCrm.pdf
4. **NCIS medical oncology antiemetic medicines**
   https://healthservice.hse.ie/documents/7180/Supportive_Care_Antiemetics_for_inclusion_NCIS_Medical_Oncology.pdf
5. **LUME-Lung 1 primary publication**
   https://pubmed.ncbi.nlm.nih.gov/24411639/